

Subject: Larry Parker (MRN AML051) — AML (NPM1-mut/FLT3-ITD–), doing well on 7+3 program

Dear Dr. Alvarez,

A quick update on **Mr. Larry Parker** (*10.10.973) who was diagnosed with AML **08.09.2023** (ELN 2022 **Favorable** risk: **NPM1** mutation, **FLT3-ITD negative**; normal karyotype). He began standard **7+3 induction** on **15.09.2023** and is currently completing post-remission therapy.

Course to date:

- **Induction (15–21 Sep 2023):** cytarabine continuous infusion + daunorubicin. He had expected pancytopenia with one episode of febrile neutropenia responsive to piperacillin–tazobactam.
- **Day-28 marrow (13 Oct 2023): Complete remission;** MRD by **NPM1 qPCR** low-positive (just above assay threshold), which cleared after consolidation cycle 2.
- **Consolidation:** high-dose cytarabine (HiDAC) at 3 g/m² q12h on days 1, 3, 5; planned **3–4 cycles** with **prophylactic steroid eye drops** and daily neuro checks. He has completed **two** cycles (Nov and Dec 2023) and **cycle 3** in Feb 2024 without neurotoxicity; conjunctivitis prevented. We continue **antimicrobial prophylaxis** (acyclovir + antibacterial during ANC <0.5); azoles avoided around cytarabine due to hepatic load.

Most recent visit: 14.03.2024

- **S:** Feeling well, walking daily, no bleeding, no fevers, appetite normal.
- **O:** ECOG 0; vitals stable; exam unremarkable; no organomegaly or petechiae.
- **Labs:** WBC 4.7 (ANC 2.0), Hgb 12.4 g/dL, Plt 212 ×10⁹/L; Cr 0.92, AST/ALT 23/21; LDH normal.
- **MRD:** **NPM1 qPCR undetectable** on peripheral blood (01.03.2024).
- **Echocardiogram:** LVEF 61% (baseline 63%).

Comorbidities/meds: Mild **hypertension** (amlodipine 5 mg qd), **GERD** (omeprazole 20 mg qAM), allergic rhinitis (cetirizine PRN). Non-smoker, no anticoagulants. Vaccinations: influenza and COVID boosters up to date.

Plan:

1. Complete **HiDAC consolidation** (cycle 4 if counts and tolerance remain good). Given sustained MRD negativity and favorable genetics, **allogeneic transplant is not indicated** at this time.
2. Continue infection prophylaxis during neutropenic windows and the usual transfusion thresholds (Hgb ≥8, Plt ≥10; ≥20 if bleeding/procedures).
3. After consolidation, transition to **surveillance**: clinic and CBC q3 months in year 1, then spacing as appropriate; **NPM1 qPCR** q3–6 months initially.
4. Counseling on fever threshold (≥38.0 °C), bleeding signs, and avoiding NSAIDs if thrombocytopenic.

Bottom line: Mr. Parker remains in remission with clean MRD and good performance status while completing planned 1L therapy.

Please reach out if any intercurrent issues arise or if you notice unexplained bruising or infections between visits.

Warm regards,
[Name], MD
Hematology/Oncology
Direct: [redacted]